

COGNITIVE BEHAVIORAL THERAPY (CBT) – BOOSTER SESSION 1

New Case: Rohan (26 y/o) – Contamination OCD + Checking

Highly Detailed – Realistic Style

SESSION INFORMATION

Client:	Rohan (26 years, Male, Software Engineer, Bangalore)	Therapist:	Dr. A.
Session Type:	Booster Session 1 (6 weeks post-termination)	Focus:	Review Progress & Maintenance

PRE-SESSION THERAPIST NOTES

This is the first booster session, 6 weeks after termination. Client has been managing independently. He practiced exposures occasionally and reviewed his Action Plan regularly. He reported one period of increased stress at work where OCD symptoms temporarily increased (more washing and checking), but he was able to use his coping skills and Action Plan effectively. Today's plan: Review progress since termination, check use of coping skills and Action Plan, address any difficulties, reinforce maintenance strategies, and confirm the next booster session (3 months). Maintain supportive and encouraging approach.

SESSION START – CHECK-IN

Therapist: Good morning, Rohan. It's good to see you again. How have you been since we last met?

Client: (speaking with noticeable confidence) I've been doing okay. There were a couple of weeks where work was very stressful, and I noticed my washing and checking increased a bit. But I used the Action Plan — I started labeling more, practiced one exposure, and reviewed my Early Warning Signs. It helped. I didn't go back to how things were before. I still feel more capable than before therapy.

Therapist: That's really good to hear. You recognized the Early Warning Signs during a stressful period and took action using your plan. That's exactly what we hoped would happen. Can you tell me more about what happened during those stressful weeks?

REVIEW OF DIFFICULT PERIOD

Client: Work was very demanding for about 10–12 days. I was sleeping less and felt more pressure. I started washing my hands more often and checking switches again at night. I noticed it after 3–4 days. I felt a bit scared that I was going backward. But then I remembered the Action Plan. I started labeling the thoughts, practiced one exposure from the hierarchy (touching door handles and delaying), and reviewed my values — especially wanting to have more energy and not feel exhausted all the time. It helped me stay on track. The symptoms didn't go away completely, but they didn't get as bad as before.

Therapist: That's excellent self-awareness and use of your tools. You caught it early, used your Action Plan, and prevented a full relapse. This shows real progress in your ability to manage OCD independently. How did it feel to handle it this way compared to before therapy?

Client: It felt very different. Before, I would have just kept washing and checking more and more without knowing what to do. This time I had a plan. I still felt anxious, but I didn't feel completely helpless. That made a big difference.

REVIEW OF COPING SKILLS & ACTION PLAN USE

Therapist: It sounds like you used several parts of your Action Plan effectively. Which coping strategies have been most helpful for you since we finished therapy?

Client: Labeling is still the one I use the most — it creates distance quickly. Urge surfing and breathing also help when anxiety spikes. I've been reviewing my Action Plan almost every day, even on good days, just to keep it fresh in my mind. I also practiced one exposure from the hierarchy every week or so, even when I wasn't feeling particularly anxious. That helped keep the skills sharp.

Therapist: That's wonderful. You're not just reacting when symptoms appear — you're also proactively maintaining your skills. That's a very strong maintenance strategy. Have there been any situations where the Action Plan didn't work as well, or where you felt stuck?

Client: Not really. There were moments when anxiety felt very strong and I wanted to wash immediately, but I was able to wait at least a few minutes using labeling and breathing. It didn't always feel easy, but it worked enough.

REINFORCEMENT & ENCOURAGEMENT

Therapist: You've handled a stressful period independently and used your tools effectively. That's significant progress. OCD often tries to come back during times of stress, but you caught it early and responded skillfully. This is exactly what long-term recovery looks like — not the absence of any symptoms, but the ability to manage them when they appear.

Client: Thank you. Hearing that helps. I still get nervous sometimes, but I feel much more equipped now.

PLANNING FOR NEXT BOOSTER

Therapist: We planned the next booster for 3 months after termination. That would be in about 6 weeks from now. Does that still feel like a good timeline, or would you prefer to come earlier?

Client: I think 3 months is fine. I feel more confident now after handling that stressful period. But I like knowing I can reach out earlier if I need to.

Therapist: That's completely fine. You have my contact information, and you can always reach out if you need support before the next booster. We'll confirm the date closer to the time.

SESSION SUMMARY & FEEDBACK

Therapist: Today we reviewed how you've been managing since termination, including a stressful period where you successfully used your Action Plan. You've shown real independence and skill in maintaining your progress. How are you feeling about today's session?

Client: I feel good. It was helpful to talk through what happened during the stressful weeks and see that I handled it well. I feel more confident about continuing on my own.

Therapist: That's wonderful. Any questions before we finish?

Client: Not really. I'll see you in about 6 weeks for the next booster, or earlier if needed.

POST-SESSION THERAPIST NOTES

- Client demonstrated strong independent management during a period of increased work stress.
- Successfully used Early Warning Signs recognition + Action Plan to prevent full relapse.
- Consistent use of labeling, urge surfing, and proactive exposure practice noted.
- Client expressed increased confidence in long-term maintenance.
- Next booster session confirmed for approximately 6 weeks from now (3 months post-termination).

Disclaimer: This is a completely fictional but realistic simulation created solely for educational and AI training purposes. It does not represent any real person or therapy.